AI reinvention made real

Accenture Innovation Challenge 2026

Team details

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PatientTriage.ai

AN AI-POWERED EMERGENCY DECISION-SUPPORT PLATFORM

that **prioritizes** patients, continuously **reassesses** risk, and intelligently **allocates** scarce ED resources.

- Faster & smarter prioritization
- Continuous reasssessment of risk
- Intelligent resource allocation
- Better utilization of scarce resources
- Safer waiting, better outcomes
- Human-in-the-loop & clinician control

WHO WE SERVE

Primary Users:

ED Nurses & ED Doctors

Across all types and sizes of hospitals in India

WHAT WE DO

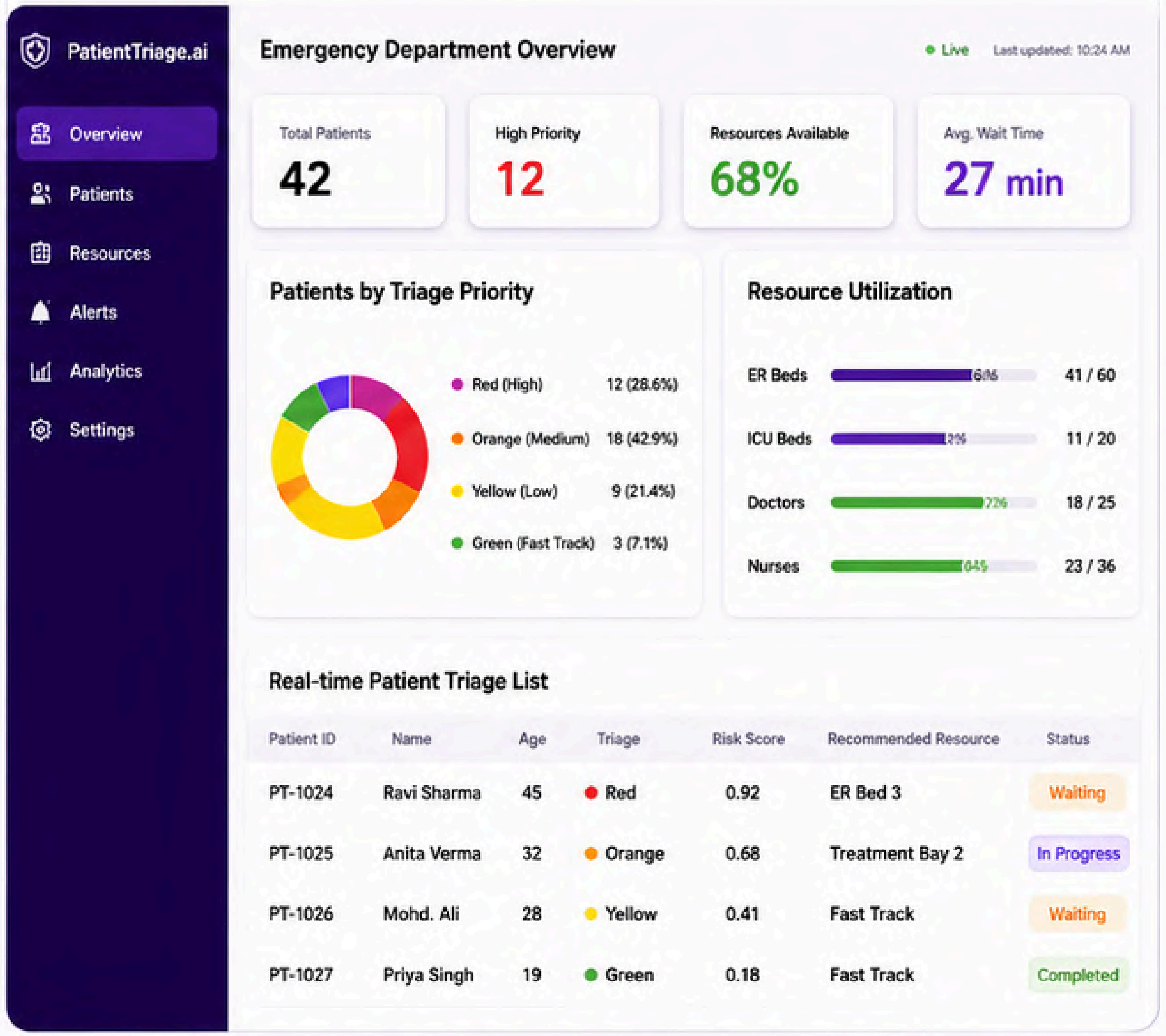
Intelligent Intake → AI Risk Assessment → Triage Prioritization → Continuous Monitoring → Resource Allocation

Right patient, Right time, Right resource

BUSINESS IMPACT

- Optimized resource utilization
- Reduced waiting time for critical patients
- Improved patient safety
- Better operational efficiency
- Data-driven decision making

LIVE DASHBOARD PREVIEW



Turning emergency data into real-time decisions — for smarter EDs and better patient outcomes.

OUR VISION

To become the most trusted AI partner for emergency care in India, helping every hospital deliver faster, safer, and smarter care.

- Works across hospitals of all sizes & specialties
- Configurable for local workflows & resources
- Secure, Compliant & Privacy Protected
- Built for Clinicians, Backed by AI

THE PROBLEM: INDIAN EMERGENCY DEPARTMENTS

OVERWHELMED. UNDER-RESOURCED. UNPREDICTABLE.

Indian EDs handle 100 – 500+ patients every day.
Staff shortages, long waits, and changing conditions put lives at risk.

1 STAFFING SHORTAGE

- High patient-to-staff ratio
- Burnout and fatigue
- Limited time for thorough assessment

India has ~1 doctor per 1,200 people (vs WHO norm of 1:1000) – NITI Aayog, 2023

2 STATIC, ONE-TIME TRIAGE

- Triage happens once, at arrival
- Patient condition can change while waiting

Up to 25% of patients may deteriorate while waiting – Studies

3 NEW PATIENTS, NEW INTERRUPTIONS

- Unpredictable arrival of new emergencies
- Disrupts workflow and delays ongoing care

ED volume variability can be 2–3x during peak hours – ICMR

4 INCOMPLETE INFORMATION

- Patients may not know their full medical history
- Missing data leads to uncertainty in decisions

~30–40% of ED visits lack complete medical history – Research

5 LONGER WAITS = HIGHER RISK

- Longer waiting times increase risk of deterioration
- Not visible until it's too late

Every 30-min delay in treatment increases mortality risk by 7% – Systematic Reviews

6 LACK OF STANDARDIZED PROTOCOLS

- Different hospitals, different triage practices
- Inconsistent prioritization leads to variability in care

No universal triage standard across hospitals in India

"It's not just about who arrives first, but who needs help the most, right now."

EDs need more than a one-time triage.

They need a system that **continuously reassesses risk** and **intelligently allocates scarce resources— in real time.**

THE COST OF INACTION:

- Delayed treatment
- Poor patient outcomes
- Higher healthcare costs
- Overburdened healthcare staff
- Erosion of trust in the system

WHY EXISTING TRIAGE ALONE IS NOT ENOUGH

TRIAGE IDENTIFIES WHO NEEEDS HELP FIRST.
EDs ALSO NEED TO KNOW WHERE AND HOW TO USE SCARCE RESOURCES—IN REAL TIME.

LIMITATIONS OF ONE-TIME, STATIC TRIAGE

PATIENT CONDITION CHANGES
Risk level can increase or decrease while waiting.

Low Risk at Arrival → High Risk 30 min Later

RESOURCES ARE DYNAMIC
Bed availability, staff, and equipment change constantly.

Bed Available Now → No Bed Available Later

NEW PATIENTS CONTINUE TO ARRIVE
EDs are a continuous inflow system, not a closed queue.

INCOMPLETE INFORMATION
Critical data may be missing initially but becomes available later.

→ More Info Becomes Available

LACK OF STANDARDIZATION
Different hospitals use different triage protocols and priority definitions.

Different Protocols → Different Priorities → Different Outcomes

THE CONSEQUENCE

Longer waits for critical patients → Higher risk of deterioration → Suboptimal use of scarce beds and staff → Higher costs & operational inefficiency → Poorer patient experience & outcomes

TRIAGE VS. WHAT EDs ACTUALLY NEED

CURRENT APPROACH	WHAT EDs NEED
One-time assessment at arrival	Continuous reassessment of risk
Focus on patient priority	Focus on right patient, right resource, right time
Ignores waiting time and deterioration	Tracks waiting time and detects deterioration early
Does not adapt to resource constraints	Adapts to real-time resource availability and capacity
Protocol variability across hospitals	Standardized core engine with hospital-specific configuration

“EDs need an intelligent system that continuously reassesses risk and dynamically allocates the right resources—so the right patient gets the right care at the right time.”

EDs need more than a static triage list.
They need a dynamic, data-driven decision-support layer.

- Continuous reassessment
- Real-time resource awareness
- Faster, safer decisions
- Better outcomes for more patients

PatientTriage.ai Business Proposal

PatientTriage.ai
Intelligence. Prioritized.

OUR SOLUTION

PatientTriage.ai — From Triage to Intelligent ED Operations

An intelligent decision-support layer that turns patient data into safer, explainable triage and real-time ED resource decisions.

1. **PATIENT ARRIVES** — Symptoms + vitals + history
2. **AI INTAKE & RISK ASSESSMENT** — Structured clinical inputs
3. **TRIAGE PRIORITY + CONFIDENCE** — Severity + uncertainty
4. **RESOURCE ALLOCATION** — Beds + chairs + staff
5. **CONTINUOUS REASSESSMENT** — Waiting time + deterioration

Dynamic & ongoing, not a one-time score

AI-ASSISTED INTAKE

New Patient Intake & Triage Engine

Natural Language Intake Parser — Powered by Gemini AI

58 yr old male, chest pain radiating to left arm, sweating, dizziness, HR 110, BP 140/90, SpO2 94%

Parse with AI

Structured Clinical Data

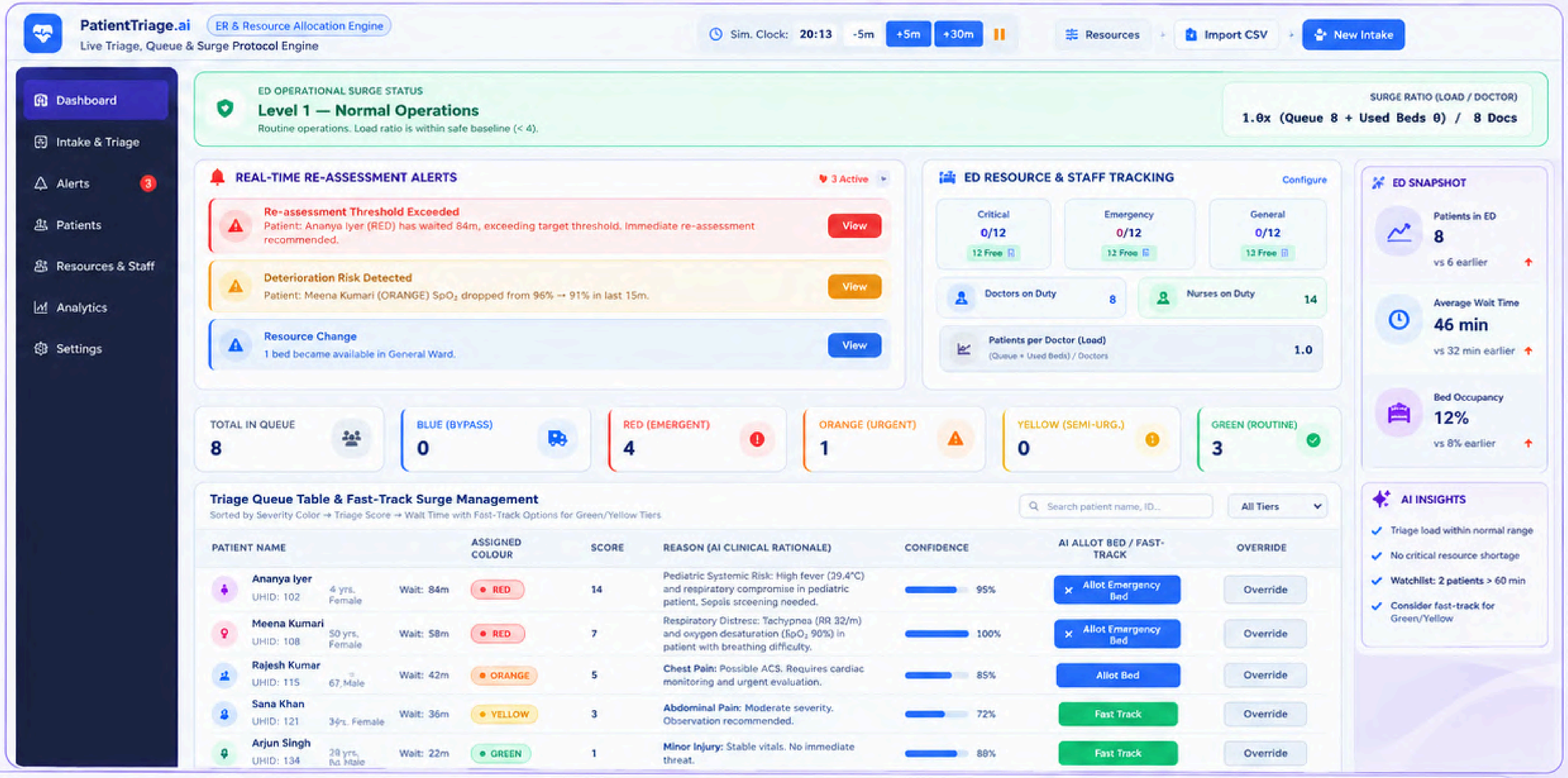
Symptoms / Chief Complaint: Chest pain radiating to left arm, sweating, dizziness

Vital Signs: BP 140/90 · HR 110 · RR 24 · SpO2 94% · Temp 36.8°C

Patient Details

Age 58 · Gender Male · History Diabetes, HTN

Run Triage & Add to Queue



PRIORITIZE — Identify who needs attention first

OPTIMIZE — Match patients to available ED resources

REASSESS — Detect changing risk while patients wait

AI recommends. Clinicians decide.

PatientTriage.ai
Intelligence. Prioritized.

A 4-STEP SCORING HIERARCHY, NOT A SINGLE SCORE

Every decision is explainable.
Every patient is protected.

1 STEP 1 — IMMEDIATE LIFE-THREAT BYPASS (BLUE TIER)

Checks first, before any point-scoring: keywords like arrest, CPR, unconscious/no pulse, active seizure, uncontrolled bleeding — OR AVPU = Unresponsive

If matched: instantly assigned Score 12, Blue Tier (Resuscitation), no further calculation needed.

2 STEP 2 — AGE-SPLIT PHYSIOLOGICAL SCORING

ADULT (AGE ≥ 12)
Points scored on deviation of RR, SBP, HR from normal ranges (e.g., RR ≤8 or ≥30 → +3 points)

PEDIATRIC (AGE <12)
6 distinct age brackets (Neonate to Adolescent) with their own normal ranges; deviation magnitude drives points (minor/moderate/severe = +1/+2/+3)

This directly solves the PS's callout that a single adult-calibrated model is a silent safety risk.

3 STEP 3 — UNIVERSAL VITALS

SpO_2, Temperature, and AVPU consciousness state scored the same way regardless of age (e.g., SpO_2 ≤91% → +3 points)

4 STEP 4 — SYMPTOM-SPECIFIC PATHOLOGY OVERRIDES

Keyword scan on chief complaint forces a tier regardless of raw score:

- Chest pain + radiation to jaw/shoulder/arm + sweating → Red (ACS pathway)
- Stroke keywords (weakness, droop, slurred speech) → Red (Stroke pathway)
- Respiratory distress keywords (SOB, wheezing, asthma) → Orange

HANDLING PATIENTS WITH VS. WITHOUT PRIOR HISTORY

WITH PRIOR RECORD (RETURNING PATIENT)

If a prior medical record / discharge summary is available, it's marked in the system and factored into the assessment (e.g., a flagged prior cardiac event raises attention on a related complaint)

Uses discharge summary to enrich risk understanding.

WITHOUT PRIOR RECORD (FIRST-TIME PATIENT)

If no record exists, the engine scores purely on the vitals and symptoms observed at that moment — no assumptions are made about missing history.

Purely evidence-based at the point of care.

Demonstrated in the prototype with a dedicated sample patient case for each scenario.

FINAL TIER MAPPING

TIER	TRIGGER	TARGET WAIT
RED (EMERGENT)	Score ≥ 7 OR max single-vital score ≥ 3 OR symptom red flag	10 min
ORANGE (URGENT)	Score 5 – 6 OR symptom orange flag	30 min
YELLOW (SEMI-URGENT)	Score 3 – 4	60 min
GREEN (ROUTINE)	Score ≤ 2	240 min

CONFIDENCE FORMULA

CONFIDENCE %

= 100 – (missing vitals × 10) – penalties

EXTRA DEDUCTIONS

–5%	For pediatric cases
–15%	For clinical tier mismatches

Below 80% confidence → warning badge (⚠) flagged for mandatory clinician verification.

SAFETY FIRST
High-risk patients never wait.

AGE-SENSITIVE
Different norms for adults vs. pediatrics.

EVIDENCE-DRIVEN
Vitals + symptoms + clinical intelligence.

TRANSPARENT & AUDITABLE
Every decision is traceable and accountable.

COMPLIANT BY DESIGN
Aligned with India's DPDP Act for patient data protection.

FROM TRIAGE TO RESOURCE ALLOCATION

Knowing who needs attention is only half the problem — the ED must also know where to allocate its limited resources.

PatientTriage.ai
Smarter Triage. Safer Outcomes.
Better Care for All.

PatientTriage.ai connects patient priority with REAL-TIME ED capacity.

It does not simply answer: "Who is most urgent?"

It also answers: "What resource should be allocated, based on current availability?"

1 PATIENT PRIORITY

- RED
- ORANGE
- YELLOW
- GREEN

Clinical severity + symptoms + vitals + uncertainty

2 CURRENT ED CAPACITY

Beds | Doctors | Nurses

Live available capacity

3 RESOURCE MATCHING

AI

Best-fit available resource

Priority × urgency × capacity

4 ACTION

- Emergency Bed
- General Bed
- Chair / Fast Track

Actionable recommendation for ED staff

PATIENT STREAM

Patient Arrives → Symptoms & History → Vitals & Observations → Severity Score & Confidence → TRIAGE PRIORITY

ED CAPACITY STREAM

ED Resources Configured → Free Beds (Real-Time) → Staff On Duty (Doctors / Nurses) → Current Load & Utilization → AVAILABLE CAPACITY

PATIENTTRIAGE.AI
AI-Powered Decision Engine

RESOURCE RECOMMENDATION

LIVE ED CAPACITY

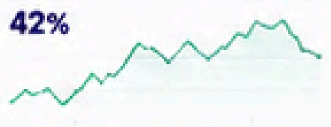
PatientTriage .ai ER & Resource Allocation Engine
Live Triage Queue & Surge Protocol Engine

Sim. Clock: 20:23 | -5m | +5m | +30m | H | Resources | Import CSV | New Intake

ED OPERATIONAL SURGE STATUS

Current Status: **NORMAL**
Surge Level: **GREEN**

ED Load
42%



Last 12 Hours

ED RESOURCE & STAFF TRACKING

BED AVAILABILITY

Critical / Resus Beds	12 / 12
Emergency Beds	12 / 12
General Beds	12 / 12
Chairs	12 / 12

STAFF ON DUTY

Doctors on Duty 8 / 8 Available

Nurses on Duty 14 / 14 Available

LIVE ED TRIAGE QUEUE

Patient Name	Severity	Score	Wait Time
Ananya Iyer	RED	14	92m
Meena Kumari	RED	7	66m
Rajesh Kumar	RED	2	58m
Sunita Rao	RED	0	44m
Vikram Malhotra	ORANGE	2	50m
Amit Patel	GREEN	2	81m
Priya Sharma	GREEN	0	216m
Karan Verma	GREEN	0	146m

Current availability feeds the recommendation

EXAMPLES (ILLUSTRATIVE)

RED Patient (High Risk) → Recommended: Emergency Bed

GREEN Patient (Low Risk) → Recommended: Chair / Fast Track

CONFIGURABLE HOSPITAL CAPACITY

Hospital Resource & Staffing Setup
Configure capacity, free/used beds, and active medical staff

BED & SEATING ALLOCATION (TOTAL & FREE)

	Total	Free
Critical / Resus Beds	12	12
Emergency Beds	12	12
General Beds	12	12

MEDICAL STAFF ON DUTY

Doctors on Duty	Nurses on Duty
8	14

Save Resource Configuration

Adapts to each hospital's beds, staffing and operational setup.

WHY THIS MATTERS

1 NO STATIC BED ASSIGNMENT
Resource recommendations reflect current availability.

2 NO ONE-SIZE-FITS-ALL
Hospital capacity and staffing can be configured.

3 ACTIONABLE TRIAGE
The output is an operational action, not just a risk score.

Built for real-world EDs where things change every minute.

Dynamic. Connected. Actionable.

AI RECOMMENDS THE BEST AVAILABLE PATHWAY — CLINICIANS RETAIN FINAL AUTHORITY.

Human-in-the-Loop | Transparent Rationale | Clinician Override | Continuous Reassessement | Better Outcomes

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SURGE MANAGEMENT

Intelligent response when demand exceeds normal ED capacity.

Built to perform under pressure.
Designed to handle a **3× surge scenario.**

1 SURGE RATIO (CALCULATED CONTINUOUSLY)

Surge Ratio = (Queue Length + Occupied Beds) ÷ Active Doctors on Duty

Recalculated every minute using real-time ED data.

2 4 SURGE LEVELS WITH DISTINCT COLOR CODING & STAFF RESPONSE

NORMAL (< 4)	MODERATE (4 – 7)	CRITICAL (8 – 11)	SEVERE / MASS CASUALTY (≥ 12)
Routine Operations	Elevated Demand	High Strain	Extreme Strain
Standard staffing & workflows	Increase monitoring & flexible staffing	Activate surge protocols & redeploy staff	Full surge response & command mode

Higher Surge Ratio → Higher Demand → Stronger Response

3 FAST-TRACK EXPRESS LANE (TRIGGERED DURING SURGE WHEN BEDS ARE FULL)

When beds are fully saturated during a surge, Green/Yellow patients are automatically routed to an ambulatory consult lane.

MAIN ED QUEUE
For Red / Blue
(High Acuity)
Acute beds reserved for Red & Blue patients

BEDS STATUS
FULL
(100% Occupied)

FAST-TRACK EXPRESS LANE
For Green / Yellow
(Low Acuity)
Ambulatory consult by Nurses / Clinical Staff
(No bed required)

OUTCOME
- Frees acute beds for critical patients
- Reduces wait for low-acuity cases
- Improves overall ED flow

Ensures high-acuity patients are not delayed by non-urgent cases.

4 QUEUE DESIGN — OPTIMIZED FOR WORST-CASE SCENARIO

Our queue is already built for the worst-case scenario. Therefore, queue structure does not change during a surge.

5 ENHANCED REASSESSMENT DURING SURGE (PROPOSED)

During surge, room for error increases. We shorten reassessment intervals to detect change earlier and reduce risk.

NORMAL	SURGE MODE
Reassessment Interval Every 30 Minutes	Reassessment Interval Every 20–25 Minutes

Faster reassessment → Earlier intervention → Safer outcomes

SURGE MANAGEMENT ENSURES SAFE, INTELLIGENT, AND SCALABLE ED OPERATIONS.

- Protects critical patients by prioritizing resources
- Maintains ED flow even under extreme load
- Empowers staff with real-time decision support
- Built to handle 3× surge scenarios

EMERGENCY

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SAFE AI. CLINICIAN IN CONTROL.

Designed to support clinical decisions — never replace them.

CLINICIAN CONTROL

- AI RECOMMENDS
- ↓
- CLINICIAN REVIEWS
- ↙ ACCEPT
- ↘ OVERRIDE — Documented reason + audit trail

OVERRIDE (EXAMPLE)

Override Triage Decision

Current AI Recommendation	RED	Emergency Bed
Override To	ORANGE	General Bed
Additional Notes Optional	Symptoms improving; stable vitals.	

Cancel | Submit Override

PATIENTTRIAGE.AI — AI DECISION SUPPORT

1. **ASSESS** — Symptoms • vitals • history
2. **EXPLAIN** — Risk + rationale + confidence
3. **REVIEW** — Clinician validates or overrides
4. **REASSESS** — Risk can change while waiting

WHEN UNCERTAIN → ESCALATE / REVIEW

Low confidence should trigger safer review — not a silent downgrade.

EXPLAINABLE RECOMMENDATION

PatientTriage.ai — ER & Resource Allocation Engine
Live Triage Queue & Surge Protocol Engine
Sim. Clock: 20:23 | -5m | +5m | +30m | Resources | Import CSV | + New Intake

Ananya Iyer
UHID: 102 • 4 yrs, Female
Arrived: 92m ago

TRIAGE SUMMARY

Assigned Colour	
RED	Emergency
Triage Score	14 / 20
Confidence	95 %

AI CLINICAL RATIONALE

Pediatric Systemic Risk: High fever (39.4°C) and respiratory compromise in pediatric patient. Sepsis screening needed.

High Fever (39.4°C) · Tachypnea · Respiratory Distress · Pediatric Risk · Potential Sepsis

AI RECOMMENDATION

Allot Emergency Bed

PATIENT SNAPSHOT

Vitals

Temp	39.4 °C ↑
RR	32 /min ↑
SpO_2	90 % ↓
HR	128 /min ↑
BP	96 / 62 mmHg

Chief Complaint
High fever, fast breathing, lethargic

History
No significant past history

WHY THIS PRIORITY? | CONFIDENCE VISIBLE

NO SILENT DOWNGRADE
Uncertainty favours safer review.

NO BLACK BOX
Recommendation comes with rationale + confidence.

NO LOSS OF AUTHORITY
Clinicians retain final decision control.

PATIENT DATA PROTECTION
Privacy, access control and auditability built into deployment.

AI RECOMMENDS. CLINICIANS DECIDE.

BUSINESS MODEL & VALUE PROPOSITION

Turn better ED decisions into measurable operational value.

WHY HOSPITALS ADOPT

VALUE TO THE HOSPITAL

1 RESOURCE UTILIZATION
Use Existing Capacity Better
Match patient priority with available beds, chairs and staff.

2 ED FLOW
Reduce Operational Bottlenecks
Help staff prioritize patients and route them through appropriate pathways.

PatientTriage.ai

3 SAFER WAITING
Catch Changing Risk
Continuous reassessment helps identify patients whose condition worsens while waiting.

4 STAFF DECISION SUPPORT
Support, Not Replace, Clinicians
Explainable recommendations reduce decision burden while keeping clinical authority with staff.

More efficient use of existing ED resources — without adding another specialist triage role.

HOW PATIENTTRIAGE.AI SCALES AS A BUSINESS

PROPOSED BUSINESS MODEL

Proposed model

1 HOSPITAL DEPLOYMENT
PatientTriage.ai deployed as an ED decision-support layer

2 RECURRING SOFTWARE
Hospital subscription / licensing

3 IMPLEMENTATION & INTEGRATION
Initial integration, configuration and onboarding

PILOT HOSPITAL → MULTI-SITE HOSPITAL GROUP → REGIONAL SCALE → PAN-INDIA ADOPTION

Validate | Integrate | Standardize | Scale

VALUE CREATION LOOP

BETTER PRIORITIZATION → BETTER RESOURCE UTILIZATION → BETTER ED FLOW → GREATER HOSPITAL VALUE → MORE ADOPTION

Value compounds as deployment scales across EDs.

WE SELL DECISION INTELLIGENCE — NOT JUST A TRIAGE SCORE.

An intelligent layer that helps hospitals make better use of the capacity they already have.

PatientTriage.ai Business Proposal

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REGULATORY JURISDICTION & DATA PROTECTION

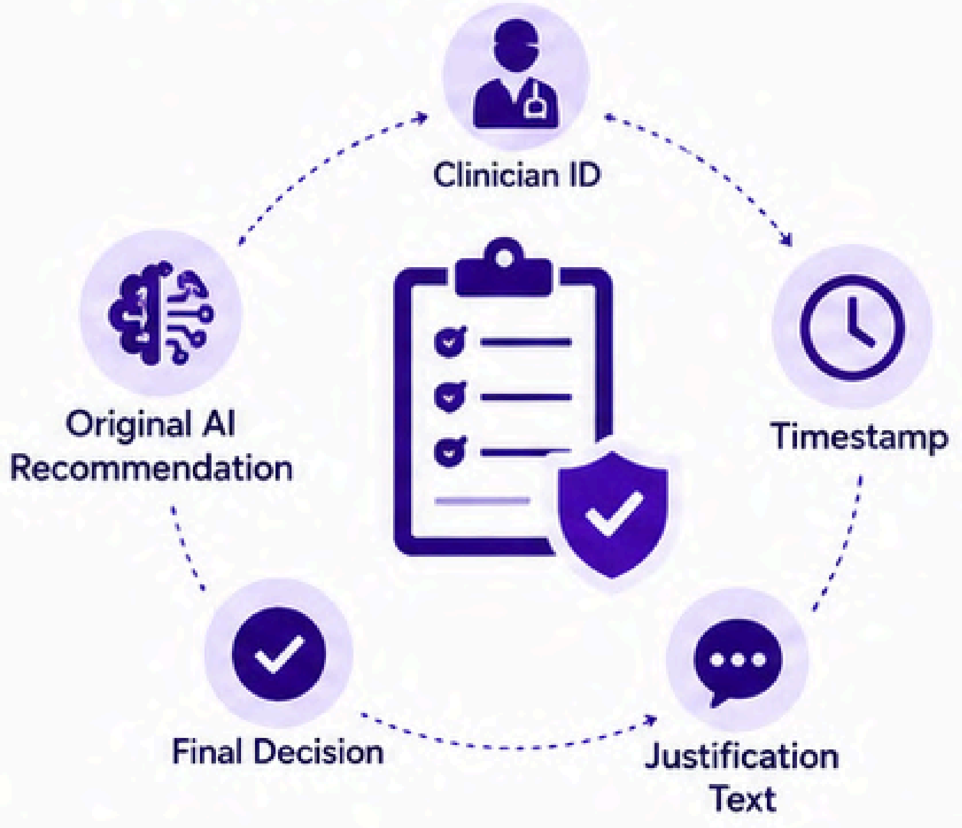
Compliance by Design — India's DPDP Act

Patient data is protected. Care is never compromised.

JURISDICTION	India's Digital Personal Data Protection Act (DPDP Act, 2023)
CONSENT	No prior consent required at intake — ED visits are processed under DPDP's medical-emergency exception, since triage is by definition time-critical and often involves patients unable to consent.
RETENTION DURING STAY	Full record kept — all vitals, complaint text, scores, and override logs — for the duration of the hospital visit, supporting real-time care and audit.
RETENTION AFTER DISCHARGE	Raw visit data is deleted; the system generates and retains a discharge summary containing clinically significant flags (e.g., prior cardiac event, chronic conditions) — enough for a future ED visit to recognize the patient's risk history without holding the full original record. This is also what powers the "returning patient" side of zero-history handling (Slide 6/14) — the discharge summary from a prior visit is the record a returning patient shows up with next time.
PURPOSE LIMITATION	The discharge summary exists solely to inform future consultations/triage — not stored for any other use.

OVERRIDE AUDIT TRAIL

Clinician ID, timestamp, original AI recommendation, final decision, and justification text — retained for the same period as the discharge summary, since it's the accountability record.



PATIENT PRIVACY. CLINICAL INTEGRITY. LEGAL COMPLIANCE.
Designed to protect patients and empower clinicians — every step, every time.

- Legal Compliance by Design
- Minimum Data Retention
- Secure Storage & Encryption
- Transparent & Auditable

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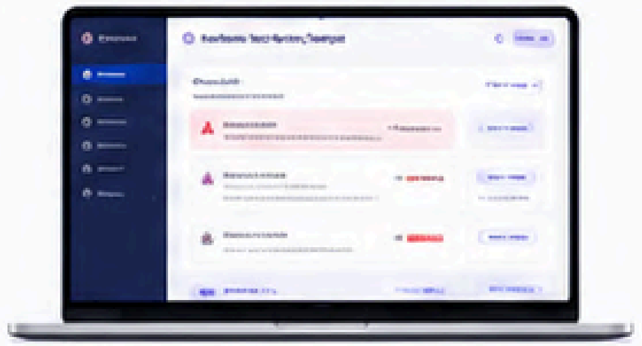
FROM PROTOTYPE TO PAN-INDIA SCALE

A phased rollout designed to validate safety, fit hospital workflows, and scale responsibly.

01 • PROTOTYPE

Validate the Core Engine



- Simulated patient records
- Ambiguous + pediatric/ geriatric cases
- Confidence + clinical rationale
- Surge simulation

Current stage

02 • CONTROLLED PILOT

Test With ED Teams

- Limited hospital deployment
- Nurse + ED doctor feedback
- Clinician override workflow
- Workflow fit assessment

Validate in real workflow

03 • HOSPITAL INTEGRATION

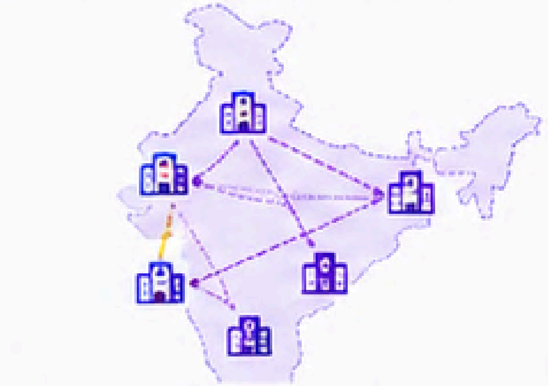
Connect to Existing Systems

- Patient records
- Bed/resource availability
- Staff rosters
- Role-based access
- Audit trail

Adapt to hospital maturity

04 • NETWORK SCALE

Scale Across Hospital Networks



- Multi-site deployment
- Configurable thresholds
- Hospital-specific workflows
- Continuous monitoring
- Regional → Pan-India expansion

Broader adoption

PAN-INDIA

Configurable across:

- Hospital size
- Specialty mix
- Staffing levels
- Technical maturity

GATE BEFORE SCALE

- **SAFETY** — Escalation + uncertainty behaviour
- **CLINICAL FIT** — Clinician feedback + overrides
- **SYSTEM FIT** — Integration + workflow reliability
- **ADOPTION** — Training + sustained usage

Each phase advances only after the previous stage demonstrates sufficient safety, workflow fit and operational readiness.

ONE CORE ENGINE — CONFIGURED FOR DIFFERENT EDs

Large Urban Trauma Centre	Mid-size Hospital	Small / Resource-Constrained ED

Configurable workflow, thresholds and resource setup.

START SMALL → VALIDATE → INTEGRATE → SCALE

Scale only when the system is safe, explainable and operationally trusted.

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KEY RISKS — AND HOW WE DESIGN AROUND THEM

In emergency care, uncertainty is unavoidable. **The system is designed to fail safely rather than optimise for average-case accuracy.**

RISKS	WHY IT MATTERS	PATIENTTRIAGE.AI SAFEGUARDS	SAFER OPERATIONAL RESPONSE
1 AMBIGUOUS PRESENTATIONS	Symptoms vary widely and may be under-reported, leading to misclassification.	**UNCERTAINTY-AWARE ESCALATION** Use multiple inputs; low confidence → review / escalate.	Symptoms + Vitals + History → Risk Assessment → Confidence → Review if Uncertain
2 ONE THRESHOLD ≠ EVERY PATIENT	Children, adults and seniors have different normal ranges and clinical interpretations.	**AGE-AWARE DECISION LOGIC** Adjust interpretation by age group, not one-size-fits-all.	PEDIATRIC / ADULT / GERIATRIC
3 INCOMPLETE / UNEVEN DATA	History may be missing or sparse, increasing uncertainty and risk of under- or over-triage.	**NO-HISTORY ≠ NO-RISK** Use available observations and account for missing data and uncertainty.	RICH HISTORY → AVAILABLE DATA → ASSESSMENT FIRST-TIME PATIENT → LIMITED DATA → LOWER CERTAINTY → REVIEW / REASSESS
4 STATIC TRIAGE CAN BECOME OUTDATED	Patient condition can worsen during long waits or ED surges.	**CONTINUOUS REASSESSMENT** Trigger re-assessment when wait time or vitals/severity worsen.	INITIAL ASSESSMENT → WAITING → CONDITION CHANGE → REASSESS → UPDATED PRIORITY TRIAGE IS A LOOP — NOT A ONE-TIME SCORE.
5 AI ERROR / CLINICAL DISAGREEMENT	AI recommendations can be wrong, incomplete or not context-appropriate.	**CLINICIAN OVERRIDE + AUDIT TRAIL** Clinicians can override and system records reason.	AI RECOMMENDS → CLINICIAN REVIEWS → ACCEPT / OVERRIDE → DOCUMENT REASON

OTHER DEPLOYMENT RISKS

- **HOSPITAL VARIATION** Configurable workflows and resource setup.
- **SYSTEM INTEGRATION** Decision-support layer around existing systems.
- **DATA PROTECTION** Role-based access and auditability.
- **ADOPTION** Training, explainability and feedback loops.

WHEN THE SYSTEM IS LESS CERTAIN, IT SHOULD BECOME MORE CAUTIOUS — NOT MORE CONFIDENT.
Safety-first design prioritises escalation, clinician review and reassessment over blind optimisation.

UNCERTAINTY → ESCALATE → CLINICIAN REVIEW → REASSESS

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THE OUTCOME: A SMARTER, SAFER ED

From a one-time triage decision to a continuously monitored operational decision-support system.

TRADITIONAL ED FLOW

- High patient volume
- Limited staff attention
- Incomplete information
- Static initial assessment
- Resource constraints

PATIENT ARRIVES → INITIAL TRIAGE → WAIT → CONDITION MAY CHANGE → REASSESSMENT

FROM STATIC TRIAGE TO DYNAMIC ED INTELLIGENCE

WITH PATIENTTRIAGE.AI

PATIENT ARRIVES → AI-ASSISTED INTAKE → RISK + CONFIDENCE → CLINICIAN REVIEW → RESOURCE ALLOCATION → CONTINUOUS REASSESSMENT

Risk can be updated as waiting conditions change.

Symptoms + Vitals + History → Risk Assessment → Confidence → Review if uncertain

AI RECOMMENDS. CLINICIANS DECIDE.

WHAT SUCCESS LOOKS LIKE

Pilot success metrics — to be measured

1. WAIT-TIME VISIBILITY
Measure: Time from arrival → triage → reassessment
Purpose: Identify patients waiting beyond safe thresholds.

2. TRIAGE FLOW
Measure: Priority distribution + queue movement
Purpose: Understand whether patients are being routed appropriately.

3. RESOURCE UTILIZATION
Measure: Bed / chair / staff allocation patterns
Purpose: Assess whether available ED capacity is being used effectively.

4. CLINICIAN TRUST
Measure: Override frequency + documented reasons
Purpose: Understand where AI recommendations need review or refinement.

5. SAFETY & REASSESSMENT
Measure: Reassessment triggers + escalation behaviour
Purpose: Ensure changing risk is surfaced rather than silently ignored.

THE GOAL IS NOT MORE AI.
THE GOAL IS BETTER ED DECISIONS, MADE FASTER — AND REVISITED WHEN CONDITIONS CHANGE.

PatientTriage.ai connects triage, clinical reasoning, resource allocation and continuous reassessment into one decision-support layer.

ONE ED → HOSPITAL → HOSPITAL NETWORK → PAN-INDIA

ONE CORE ENGINE. CONFIGURED FOR DIFFERENT EDs.
Adaptable to hospital size, staffing, specialty mix and technical maturity.

PATIENTTRIAGE.AI | PRIORITIZE BETTER. ALLOCATE SMARTER. REASSESS CONTINUOUSLY. | AI recommends. Clinicians decide.

Thank you

